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# The Indian Hair Loss Playbook

Diagnose it properly. Treat what actually works.
Stop paying for what does not.

The shedding audit / the eight conditions / the blood tests worth paying for / minoxidil, finasteride and microneedling with the real trial numbers / PRP, lasers and transplants at 2026 Indian prices / the claims the ASCI has already struck down / ten printable worksheets

# Before you read this

This book exists because hair loss is the one common Indian health problem where the cheapest effective treatment costs about Rs. 250 a month and the most expensive treatment costs about Rs. 3,00,000 — and almost nobody tells you which one you actually need. The internet is full of before-and-after photographs and short of numbers. So this book is built on numbers: trial results, Indian clinic data, official guidelines, and 2026 market prices you can check yourself.

It is written for the person who has been losing hair for four months and has already bought one serum. It is also written for the woman whose doctor said “it is only stress” and sent her home, and for the man who is three clicks away from booking a transplant he may not need yet.

## How to use it

Read Part One and do the seven-day audit before you spend another rupee. Read Part Two with your dermatologist in mind — it is written so you can ask informed questions, not so you can self-prescribe. Use Part Three as a routine and Part Four as a budget. Print Part Five.

## What this book is not

It is not a prescription, and it is not a diagnosis. Nothing here replaces a clinician who has examined your scalp, knows your medical history and your other medicines, and takes responsibility for the decision. Every dose and drug in this book belongs in a conversation with a registered doctor. What this book does is make that conversation shorter, sharper and much cheaper for you.

> **A word on honesty**
>
> Where the evidence for a treatment is strong, this book says so. Where it is thin, it says that too, even when the treatment is expensive and popular. Where a number is a market price range rather than a published statistic, it is labelled as one. You should be able to check every claim in the appendix.

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PART ONE

# Diagnose before you spend

Almost every rupee wasted on hair loss is spent before anyone knows what is actually happening on the scalp. This part gives you the same first hour a good dermatologist spends: what kind of shedding or loss you have, what the Indian data says about how common each type is, which blood tests are worth the money, and the handful of signs that mean you should stop reading and book an appointment this week.

# 1. Shedding or losing: the distinction that decides everything

Hair falling out is not one problem. It is at least eight different problems that look similar in a bathroom sink and need completely different answers. Before you buy anything, work out which one you have.

Dermatologists separate two big families. The first is **shedding** — more hairs than usual leaving the scalp each day, while the follicles themselves are healthy and still capable of growing hair. The second is **loss** — something is actively stopping hair from growing: a genetic sensitivity to hormones, an immune attack, permanent tension on the roots, or a scar. The American Academy of Dermatology puts the difference simply: shedding often stops on its own, loss does not stop until the cause stops.

This matters because the two families respond to different things. If you have pure telogen shedding from an illness, a crash diet or childbirth, no medicine will speed it up much — it resolves in six to nine months once the trigger goes. If you have pattern loss, nothing you eat, oil or shampoo will change the trajectory; only treatments that interfere with the underlying process will.

### What is normal

Shedding 50 to 100 hairs a day is normal, because new hair is growing in at the same time. You will always see more on wash day and more in the monsoon. A head with 100,000 follicles cycling through growth and rest phases is not a static object; it is a conveyor belt.

The number that matters is not the hairs on your pillow. It is whether your **parting is getting wider, your hairline is moving, your ponytail is thinner, or your scalp is becoming visible through the hair**. Those are structural changes, and they do not happen from a bad week.

> **The one-minute triage**
>
> Ask yourself four questions. (1) Am I seeing more hair fall than six months ago, or is my hair simply thinner than it was? (2) Is the change diffuse all over, or in a pattern (temples, crown, widening part)? (3) Did something happen two to four months ago — fever, surgery, delivery, a death in the family, a very strict diet, a new job with night shifts? (4) Is there itching, scaling, pain, or a patch of completely smooth skin? Diffuse plus a recent trigger points to shedding. Pattern points to androgenetic loss. Itch and scale point to the scalp itself. A smooth patch points to alopecia areata or a scar.

# 2. The seven-day audit you can do at home

You cannot judge hair fall from memory. Memory is anchored to your worst day. Seven days of written data beats six months of anxiety.

### The daily count, done properly

Pick a fixed 24-hour window — say, from the moment you wake to the moment you sleep — and collect every hair you shed in one place: on a dark towel while washing, in a mesh strainer in the shower drain, on a comb. Count once at night. Write it down. Do this for seven days including two wash days.

Interpretation is rough but useful: under 50 a day on non-wash days is unremarkable. A consistent 100 to 150 with a rising trend, especially if it doubles on wash day, is worth a dermatologist's opinion. Anything above 200 a day, sustained, is not something to watch at home.

One warning. Do not pull on your hair to test it more than once a week. A gentle pull test — grasp a small bundle of about 50 to 60 hairs near the scalp between thumb and forefinger, slide firmly along the shaft — normally releases zero to three hairs. Six or more suggests active shedding. Done daily, the test itself becomes a stressor and a self-fulfilling panic.

### The 60-second comb test

Before shampooing, comb from back to front over a towel or sink for 60 seconds, with the same comb, at the same time of day, once a week. Count the hairs. This gives you a comparable number week to week, which the daily total does not, because wash-day counts swing wildly.

### Photographs, because mirrors lie

You will not notice a 10 percent change month to month, and you will misremember what you looked like. Take four photographs on the first of every month: front hairline, mid-scalp from above, crown from above, and one temple. Same room, same light, same phone distance, hair dry and parted the same way, no styling product. Save them in a folder named by month. This is the single most valuable thing in this book, and it costs nothing.

When you compare, compare month 1 with month 4, not month 3 with month 2. Hair grows about one centimetre a month, so cosmetic change takes a season to become visible.

## 3. The eight conditions, and how each one looks

The Indian data is unusually clear about what people actually turn up with at dermatology clinics. A multicentre database study of 24,595 Indian patients with hair loss — published as the HAILO study — found telogen effluvium in 40.70 percent, female pattern hair loss in 22.09 percent, alopecia areata in 12.81 percent and male pattern hair loss in 7.49 percent. Nearly 75 percent of those patients were women, and most were between 12 and 39 years old.

That single study explains why the internet's hair-loss advice so often misses. The loudest online conversation is about male balding. The largest Indian clinic population is women with shedding.

| Condition | What you see | What drives it |
|---|---|---|
| Telogen effluvium (shedding) | Diffuse thinning all over, lots of hair on pillow and in drain, no bald patches | A trigger two to four months earlier: fever, surgery, delivery, severe stress, crash diet, new medication |
| Androgenetic alopecia, male | Receding temples, thinning crown, hairline moving back | Inherited follicle sensitivity to DHT; family history is the strongest risk factor |
| Female pattern hair loss | Widening part, thinning over the crown, frontal hairline usually preserved | Hormonal and genetic; often runs with PCOS and metabolic syndrome |
| Alopecia areata | One or more completely smooth round patches, sometimes eyebrows or beard | Autoimmune attack on the follicle; can be triggered by stress or illness |
| Traction alopecia | Thinning along the hairline or where hair is pulled, small bumps, tenderness | Years of tight buns, braids, extensions, clips, tight headbands |
| Scarring alopecia (e.g. frontal fibrosing) | Bald band at the front hairline, loss of eyebrows, shiny scalp, no visible openings | Inflammatory destruction of the follicle; permanent if not treated early |
| Seborrheic dermatitis / dandruff | Flaking, itching, redness, greasy scales, often with shedding | Yeast overgrowth and oiliness; very common in Indian humidity |
| Anagen effluvium | Rapid, heavy loss within weeks | Chemotherapy, radiation, some drugs |

### Androgenetic alopecia is more common in Indian men than most people assume

A population-based Indian study of 1,005 men aged 30 to 50 found pattern hair loss in 58 percent of them — 47.5 percent in the 30 to 35 group, 58.7 percent in the 36 to 40 group, and 73.2 percent in the 41 to 45 group. Family history is the dominant risk factor; one Indian clinic study found a family history odds ratio of 16.86 for androgenetic alopecia.

In women, a western India study of 90 patients with female pattern hair loss found a mean age of 34.7 and a meaningful association with metabolic syndrome — which is why a good clinician will ask about periods, weight, blood pressure and sugar, not just your hair.

### You can have two at once

This is where self-diagnosis fails. A woman can have early female pattern loss that nobody noticed plus telogen shedding after dengue. A man can have pattern loss plus seborrheic dermatitis that makes the shedding look worse. A teenager can have iron deficiency plus traction from a tight ponytail. If your hair does not improve on the obvious treatment after four months, the diagnosis is incomplete, not the treatment.

## 4. The blood tests worth paying for

Hair loss is one of the few conditions where patients routinely order ten tests and act on none of them. Here is a short, defensible list, and an honest account of what each result can and cannot tell you.

### Start with these five

| Test | Why it is ordered | How to read it |
|---|---|---|
| Complete blood count | Anaemia is common in Indian women and worsens shedding | Low haemoglobin with low MCV suggests iron deficiency; discuss with your doctor |
| Serum ferritin | The best single marker of iron stores | Many clinicians treat below 40 ng/mL as low in an otherwise healthy person with hair loss; WHO uses 70 ng/mL where inflammation is present |
| TSH | Both hypothyroid and hyperthyroid states cause shedding | Outside the lab range, or a rising TSH with symptoms, needs treatment |
| Vitamin D (25-OH) | Deficiency is extremely common in India and associated with several alopecias | Correct genuine deficiency; do not expect supplements alone to regrow hair |
| Vitamin B12 | Low B12 is common in vegetarian diets and after long-term metformin | Treat the deficiency, then re-check |

### Add these only if they fit your picture

- **Women with acne, unwanted facial hair, irregular periods or a widening part:** total and free testosterone, DHEAS, and in some cases 17-hydroxyprogesterone, to look for PCOS or other androgen excess.
- **Heavy periods, or a ferritin that stays low despite tablets:** iron studies, and a conversation about the source of blood loss — this is a medical question, not a hair question.
- **Smooth patches, eyelash or eyebrow loss, or a shiny scarred scalp:** a dermatologist's examination matters far more than blood work. A scalp biopsy may be needed.

- **Sudden severe shedding with weight loss, palpitations or heat intolerance:** thyroid function plus clinical review.

### The tests that are not worth it

Hair mineral analysis has no accepted role in diagnosing hair loss and is not a substitute for blood tests. Broad “hair wellness” panels that measure twenty trace elements mostly generate anxiety. IgG food-intolerance tests are not validated for hair loss. If a clinic's first move is a long panel of expensive tests before anyone has looked at your scalp with a magnifier, that is a sales process, not a diagnosis.

> **One thing to stop before your blood test**
>
> If you take biotin — sold in almost every hair, skin and nail supplement in India, often at 5 to 10 mg per capsule — stop it at least two days before blood tests, and up to a week if you take high doses. The US FDA has issued safety communications because biotin interferes with immunoassays and can produce falsely low or falsely high results for thyroid hormones, ferritin, vitamin D, troponin and others. A falsely low TSH or a falsely high ferritin can send you down the wrong road entirely.

## 5. Reading your own reports without panicking

Lab reports are reference ranges built for populations, not for you. Two numbers cause most of the confusion in hair loss: ferritin and vitamin D.

Ferritin is an acute-phase protein, which means it rises with inflammation, infection and liver disease. So a “normal” ferritin in someone with chronic inflammation can hide iron deficiency, while a ferritin of 15 in a healthy young woman with heavy periods and diffuse shedding is a real finding. A large body of clinical guidance now uses 40 ng/mL as a practical cut-off in otherwise healthy people, and 70 ng/mL where inflammation is present. One case-control study in women with telogen effluvium found an odds ratio of 21 for shedding when ferritin was 30 ng/mL or lower.

Here is the honest caveat, and you will not read it in most Indian hair-care content: the evidence that iron supplementation regrows hair in the absence of anaemia is weak. Iron is associated with shedding; it is not proven to be a cure for it. Treat genuine deficiency because deficiency is bad for you, not because a tablet is a hair-growth drug.

Vitamin D behaves similarly. Deficiency is very common in Indian patients with hair loss — one study found 79.4 percent of telogen effluvium patients deficient — but correcting it does not reliably regrow hair on its own. Correct it, then keep going with the treatment that matches your diagnosis.

A practical rule: bring your reports to the appointment with three questions written down. Which of these numbers actually changes what you will do? What is the target we are aiming for, not just the lab range? And when do we re-test? If a clinician cannot answer the first question, the test should not have been ordered.

## 6. Red flags: see a dermatologist this week, not next month

Most hair loss can wait a fortnight. Some cannot, because the follicle is being destroyed and the loss becomes permanent.

- A patch of completely smooth skin with no visible hair openings, especially if it is spreading or there are several patches.
- Loss of eyebrows, eyelashes, beard or body hair.

- A band of baldness along the front hairline, a shiny scalp, or scalp that looks scarred, pale and smooth.
- Pain, burning, tenderness, or a scalp that feels hot.
- Redness, pus, crusting, or pustules around follicles.
- Sudden, very heavy shedding — handfuls — rather than a gradual increase.
- Hair loss with fever, joint pain, mouth ulcers, unusual fatigue or unexplained weight change.
- Hair loss that began after starting a new medicine: some drugs used for blood pressure, epilepsy, acne, depression, thyroid disease and cancer treatment are associated with shedding. Never stop a prescribed medicine on your own; ask the prescriber.

  Scarring alopecias, including frontal fibrosing alopecia and lichen planopilaris, are the group where delay genuinely costs you hair you will not get back. They are not common, but they are the reason a persistent, itching, burning, shiny patch deserves an examination rather than a serum.

PART TWO

# What actually works, ranked by evidence

This part walks through every treatment a clinic in India will offer you, from a Rs. 300 bottle of minoxidil to a Rs. 3 lakh transplant, and tells you what the trials actually showed. Where the evidence is strong, it says so. Where it is thin, it says that too. Nothing here is a prescription; everything here is what you should be able to discuss with your dermatologist in plain language.

# 7. Topical minoxidil: the baseline everything else is measured against

Minoxidil is the only topical medicine approved for pattern hair loss that you can buy without a prescription, and after four decades it is still the reference point for every new treatment. It is also the most misused.

### What it does

Applied to the scalp, minoxidil prolongs the growth phase of the follicle and widens miniaturised hairs. It does not create new follicles. That is why the honest expectation is **maintenance plus modest thickening**, not a new head of hair. The American Academy of Dermatology states plainly that minoxidil can help early hair loss and cannot regrow an entire head of hair, and that results usually take 6 to 12 months.

### The two rules that decide whether it works

1. **Contact with scalp, not hair.** Part the hair in rows about an inch apart and apply along the part line, then massage gently. A millilitre spread on the surface of long hair does nothing.
2. **Every day, forever.** Minoxidil is a treatment, not a course. Stop it and the hair it was holding goes within three to six months, back to where you would have been. This is not withdrawal damage; it is simply the natural history resuming.

### Strength, frequency and formulation

Five percent is the standard for men. For women, 5 percent is now widely used; 2 percent is gentler but slower. Once-daily 5 percent is a common compromise for women who find twice daily impossible. Foam is easier for people with sensitive scalps and dries faster; solution is cheaper and easier to get to the scalp through dense hair. Both work when used properly.

Indian prices in 2026 run from roughly Rs. 250 for a basic 5 percent solution to about Rs. 1,400 for a premium 60 ml bottle; a branded 60 ml such as Fultress F carries an MRP around Rs. 761. A month's supply of the cheap version costs less than a single hair spa. If cost is the barrier, choose the cheap solution and use it correctly rather than the expensive one you skip.

### The dread shed

In the first two to six weeks some people shed more. This is the old resting hairs being pushed out as the follicles restart, and it is a sign the drug is working, not a reason to stop. Knowing this in advance is the single most useful thing in this chapter, because most people who abandon minoxidil do it in week four.

### Side effects, honestly

- Itching, flaking, redness at the application site — usually the propylene glycol in the solution. Switching to foam or a glycol-free formulation often fixes it.
- Unwanted facial hair if the product runs onto the forehead or you sleep on it; wash your hands and pillowcases, and apply at least an hour before bed.
- Dizziness or palpitations are rare with topical use and warrant stopping and asking a doctor.
- It is not for use in pregnancy or breastfeeding without a doctor's explicit instruction.

# 8. Oral minoxidil: the change nobody explained

Low-dose oral minoxidil has moved from a niche trick to a mainstream option in the last few years, and it is the biggest practical change in hair-loss treatment since finasteride. It is also off-label, which means the dose, monitoring and decision belong to a doctor.

In 2024 and 2025 an international group of 43 hair-loss specialists from 12 countries published a consensus statement on low-dose oral minoxidil. It recommended a typical starting dose of 1.25 mg daily for adult women with a range of 0.625 to 5 mg, said baseline laboratory tests and an ECG are not routinely needed in the absence of precautions, and concluded that the earliest realistic time to see benefit is three months — six months if you had transient shedding at the start. It also noted that unwanted body hair is more likely than with the topical form.

Head-to-head evidence keeps the picture balanced. A 2024 randomised trial in 90 men compared 5 mg oral daily with 5 percent topical twice daily over 24 weeks and found oral was not superior; hypertrichosis occurred in 49 percent of the oral group. A 2024 meta-analysis of four trials found similar hair density and diameter between oral and topical, with about twice the rate of unwanted hair growth on oral.

A dose-response meta-analysis found that every additional 1 mg per day was associated with roughly 47 more hairs per square centimetre at six months, but also a 17.9 percentage point rise in hypertrichosis risk and a 4.8 percentage point rise in cardiovascular adverse events. That is the honest trade: more dose, more effect, more side effects.

### Who oral minoxidil is for

It suits people who cannot tolerate topical, whose scalp application is unreliable because of long hair or a busy schedule, or who have diffuse shedding where covering the whole scalp topically is impractical. It is not a shortcut for someone who has not tried topical, and it is not appropriate without a clinician reviewing blood pressure, fluid retention, heart history and pregnancy plans.

## 9. Finasteride and dutasteride: the strongest lever for male pattern loss

If you are a man with confirmed androgenetic alopecia, this is the treatment with the best evidence for stopping progression. It is also the one with the most online fear. Both facts are real, and you deserve the numbers rather than the slogans.

### What the trials showed

In the phase III programme covering 1,879 men, one milligram of finasteride daily produced visible improvement in vertex photographs in 48 percent of men at one year and 66 percent at two years, against 7 percent on placebo. At two years, 83 percent of men on finasteride had no further hair loss compared with 28 percent on placebo. A 2010 meta-analysis of twelve trials found hair count increased by 9.42 percent in the short term and 24.3 percent in the long term versus placebo, with the number needed to treat for one man to feel improved at 3.4 in the long term.

Dutasteride, a stronger 5-alpha-reductase inhibitor, is used off-label at 0.5 mg and has good evidence in men who do not respond adequately to finasteride. It is not a first-line choice and belongs in a specialist conversation.

### The side-effect conversation, with numbers

In the pooled phase III data, sexual adverse events were reported by 3.8 percent of men on finasteride versus 2.1 percent on placebo — decreased libido 1.8 versus 1.3 percent, ejaculation disorders 1.2 versus 0.7 percent, erectile dysfunction 1.3 versus 0.7 percent. These resolved in most men who

continued and in all who stopped. The 2010 meta-analysis found an increase in erectile dysfunction with a number needed to harm of about 82.

The Indian dermatology guidelines on finasteride in androgenetic alopecia summarise the same figures and note that the effect on sexual function is minimal for most men, while acknowledging that a small subset report effects that persist after stopping, and that nocebo effects explain some of the reporting. The US FDA added labelling about persistent sexual symptoms in 2012 while stating there is no established causal link.

What this means in practice: the honest risk is small but not zero, it is discussed before you start rather than discovered afterwards, and if you experience persistent effects you should report them rather than quietly stopping and googling.

### Rules you must not break

- Women who are pregnant or may become pregnant must not handle crushed or broken tablets; the drug is contraindicated in pregnancy.
- It is not prescribed to women of childbearing potential for hair loss in routine practice, and never without a specialist.
- It does not work in postmenopausal women the way it does in men, and it is not a treatment for shedding.
- It takes four to six months before you can judge it, and it must be continued to hold the benefit.
- Blood donation: many blood banks defer donors on finasteride. Mention it.

Indian pricing is modest: generic finasteride 1 mg typically runs Rs. 100 to Rs. 400 a month, dutasteride 0.5 mg roughly Rs. 40 to Rs. 350. Compared with any clinic package, this is the cheapest high-evidence intervention available for male pattern loss.

## 10. Women: a different problem with different rules

Female pattern hair loss is the second most common diagnosis in Indian hair-loss clinics, and it is under-treated because it is mistaken for “seasonal hair fall” for years. The treatments overlap with men's, but the sequence does not.

### What to do first

Confirm the pattern. Widening part with a preserved frontal hairline is female pattern loss; diffuse thinning with a clear trigger is shedding. Then look for the drivers that are specific to women: PCOS and androgen excess, thyroid disease, iron deficiency from heavy periods, and postpartum hormonal shifts. An Indian study of female pattern hair loss found a meaningful association with metabolic syndrome, which means a clinician may reasonably check blood pressure, fasting sugar and lipids.

### Treatment options, in order

- **Topical minoxidil 5 percent** — the best-evidenced first line for women, once or twice daily. Expect 6 to 12 months.
- **Low-dose oral minoxidil** — increasingly used where topical is impractical; the consensus starting dose for adult women is 1.25 mg daily.
- **Spironolactone** — an anti-androgen used off-label, typically for women with signs of androgen excess. Needs monitoring and is contraindicated in pregnancy.
- **Combined oral contraceptive pills** — useful when the driver is androgen excess, chosen with a gynaecologist.
- **Microneedling** — as an adjunct to minoxidil, not instead of it.

- **Hair transplant** — possible in selected women with stable loss and good donor density, but far more technically demanding than in men and never a first step.

### Pregnancy and breastfeeding

Postpartum shedding is normal, common and temporary. The American Academy of Dermatology describes the pattern precisely: shedding begins about two months after delivery, peaks around four months, and hair usually regains normal fullness within six to nine months. Do not start minoxidil or any prescription treatment during pregnancy or breastfeeding without your obstetrician's agreement; focus on iron, protein, sleep and patience. If shedding is still severe at twelve months, that is a reason to be examined, not a reason to wait longer.

## 11. Microneedling: the best-value add-on in the whole field

This is the one procedure in this book where an Indian randomised trial became the global reference. It is cheap, it works as an add-on, and it is easy to do badly.

The 2013 Dhurat study from Mumbai randomised 100 men with mild to moderate androgenetic alopecia to weekly microneedling with 1.5 mm needles plus twice-daily 5 percent minoxidil, or minoxidil alone. At 12 weeks, the mean increase in hair count in the treated area was 91.4 versus 22.2 hairs per square centimetre, a statistically significant difference. Eighty-two percent of the microneedling group reported more than 50 percent improvement, against 4.5 percent of the minoxidil-only group.

A 2025 systematic review and meta-analysis of 12 randomised trials with 631 patients confirmed the direction: combined microneedling with minoxidil significantly improved hair count and hair diameter compared with minoxidil alone, with mild and self-limiting adverse events. Interestingly, needle depth, duration and device type did not significantly change the result.

### The protocol that follows from the evidence

- Clean the scalp and the device. Use a fresh, sterile roller or stamp; a shared or reused device is an infection risk.
- Work in overlapping passes — vertically, horizontally and diagonally — over the thinning area until you see mild redness, not blood.
- Once a week is the studied frequency. More is not better.
- Do not apply minoxidil on the day of needling. The Dhurat protocol resumed minoxidil 24 hours later. Applying alcohol-based solution into open micro-channels stings and adds nothing.
- Expect some redness for a day and occasional pinpoint scabs. Stop and see a doctor if you get persistent pain, pus or fever.

Cost in India: a 1.5 mm dermaroller costs Rs. 300 to Rs. 1,500, and a clinic session typically Rs. 1,000 to Rs. 3,000. Replace rollers every few months, or use a stamp that can be disinfected. If you have active acne, eczema, psoriasis or a bleeding disorder on the scalp, ask before you start.

## 12. Ketoconazole and the scalp itself

Dandruff is not a cosmetic nuisance in this story. Seborrheic dermatitis causes itch, scale and shedding, and it makes everything else look worse.

Ketoconazole 2 percent shampoo has modest, plausible anti-androgenic activity on the scalp and is a reasonable adjunct in pattern hair loss, but it will not regrow hair on its own. Where it genuinely helps is in controlling the inflammation and flaking that drive scratching and shedding. Use it two to three times a

week, leave it on for three to five minutes, and rotate with a milder shampoo if it dries your hair.

In an Indian survey of 1,200 people published in 2026, 25 percent of respondents reported dandruff, 15 percent an itchy scalp and 10 percent hair fall, and 40 percent reported at least one scalp issue attributed to their hair-oiling habits. If your scalp flakes and itches, treat the scalp first — no amount of follicle medicine works well on an inflamed surface.

## 13. PRP, GFC and the injection economy

Platelet-rich plasma is the most sold procedure in Indian hair clinics and the one with the widest gap between marketing and evidence. The evidence is not zero. It is just much weaker than the price suggests.

A 2025 systematic review and meta-analysis pooled 43 randomised controlled trials with 1,877 participants and found moderate evidence that PRP increases hair density, reduces hair loss, and improves patient-reported outcomes, with activated PRP performing better than non-activated. It found **no significant effect on hair thickness**. An earlier 2024 meta-analysis of 14 trials and 431 patients reported a mean density gain of about 27.5 hairs per square centimetre but graded the certainty of evidence as low, with high heterogeneity and likely publication bias.

Translation: PRP can add density in some people, probably on top of minoxidil, probably modestly, and the effect on thickness is unproven. It is a reasonable add-on for someone already on a proper medical regimen who wants more, and a poor first purchase for someone who has not yet tried the cheap, high-evidence basics.

### What it costs, and what to ask

Indian clinics charge roughly Rs. 3,000 to Rs. 12,000 per session, usually sold as a package of four to six sessions plus top-ups — so a realistic twelve-month spend is Rs. 12,000 to Rs. 72,000. Before you pay, ask: how is the blood processed, is it activated, how many platelets per microlitre, who performs the injection, what is the plan if there is no change at four months, and what does the package exclude?

Growth factor concentrate (GFC) and “stem cell PRP” are marketed as upgrades. The evidence base for both is thinner than for standard PRP. The Advertising Standards Council of India has previously upheld complaints against clinics claiming “stem cell” hair regrowth and “100 percent success” on the grounds that the claims were unsubstantiated. When a clinic leads with the technology name instead of your diagnosis, that tells you something.

## 14. Laser combs, LED helmets and low-level light therapy

Low-level laser therapy is real, modest, and badly served by the devices sold online. It is the clearest case in this book of “the category works, this product may not”.

A 2024 systematic review and meta-analysis of 38 studies covering 3,098 patients found that hair density increased significantly with low-level laser or LED therapy in androgenetic alopecia compared with placebo, at both under 20 weeks and beyond. But a separate 2024 meta-analysis comparing minoxidil plus laser therapy with minoxidil alone found no significant difference in hair count or diameter. In other words: LLLT may add something on its own, but it has not been shown to beat good medical treatment.

If you buy a device, check the wavelength (around 650 nm red or 655 nm), the number of diodes and the coverage area, whether there is published trial data for that exact device, and the return policy. Then give it six months of consistent use at the stated schedule. A helmet used twice and abandoned is a Rs. 15,000 ornament.

# 15. Hair transplant: candidacy, cost, and the questions that protect you

A transplant moves hair; it does not create it. Done well, on the right person, at the right time, it is the most satisfying treatment in this field. Done on the wrong person in a hurry, it is the most expensive mistake.

### Who is a good candidate

- Stable or well-controlled pattern loss, usually after at least a year of medical treatment.
- Good donor density at the back and sides of the scalp — this is the limiting factor, not your budget.
- Realistic expectations: density, not a teenage hairline.
- Age matters. A 23-year-old with rapid loss is usually better served by medical treatment first, because the pattern is still declaring itself.
- Non-smokers heal better; uncontrolled diabetes and active scalp inflammation should be managed first.

### Graft numbers and Indian costs in 2026

| Stage (Norwood) | Typical grafts | Market range in India |
|---|---|---|
| 2 to 3, temples receding | 800 to 1,800 | Rs. 40,000 to Rs. 1,80,000 (FUE) |
| 3 to 4, temples plus crown | 1,500 to 2,800 | Rs. 75,000 to Rs. 3,00,000 (FUE) |
| 5, advanced | 2,500 to 4,000 | Rs. 1,25,000 to Rs. 4,80,000 (FUE) |
| 6 to 7, extensive | 4,000 to 5,500+ | Rs. 2,00,000 to Rs. 6,60,000+ (FUE) |
| Female pattern loss | 800 to 2,500 | Rs. 40,000 to Rs. 3,00,000 (FUE) |

Per-graft pricing in 2026 runs from about Rs. 20 to Rs. 40 at the cheapest “technician-run” setups, Rs. 40 to Rs. 100 at mid-range clinics with a qualified surgeon, and Rs. 100 to Rs. 150 or more with senior surgeons in metros. A Rs. 25,000 package for 3,000 grafts works out to about Rs. 8 a graft. Ask yourself who is being paid that, and for what. Technique matters less than who performs each step: FUT, FUE, sapphire FUE and DHI all work in trained hands.

Budget for the extras that quotes often exclude: PRP sessions after surgery at Rs. 5,000 to Rs. 15,000 each, medicines, medicated shampoos, and months of continued medical treatment. A transplant without ongoing medical therapy in a man with progressive loss is a plan to look good for three years and odd for ten.

### The rules that now exist in India

The National Medical Commission's 2022 guidelines on aesthetic surgery and hair transplant are explicit: hair transplantation is a surgical procedure, and the preoperative evaluation, planning, donor harvesting, hairline design, recipient site creation and post-operative care must be performed by a registered medical practitioner. It says the procedure should preferably be done by those with formal surgical training — MCh or DNB in plastic surgery, or MD or DNB in dermatology with grooming in dermatological surgical procedures — that the facility needs a proper day-care operation theatre with emergency drugs, resuscitation equipment and a standby anaesthetist, and, pointedly, that watching workshops or YouTube videos is not adequate training.

States are now enforcing this. Tamil Nadu issued a government order in February 2026 requiring aesthetics and hair transplant clinics to register under the Tamil Nadu Clinical Establishments Act, with

procedures performed only by registered, qualified dermatologists or plastic surgeons, proper pre-anaesthetic checks, and emergency equipment on site.

### Twelve questions to ask before you book

1. Who exactly will make the incisions and who will extract the grafts? Will the doctor do both, or technicians?
2. What is your registration number with the state medical council?
3. How many grafts do you estimate, and what is the basis for that number — did you measure donor density?
4. Is the quoted price all-inclusive? List everything excluded, in writing.
5. What is the plan if graft survival is poor?
6. How many of these procedures do you personally perform each month?
7. Can I speak to two patients from twelve months ago, not three months ago?
8. What medical treatment will I stay on afterwards, and for how long?
9. What are the risks in my case specifically — shock loss, poor density, scarring, infection?
10. Is there a cooling-off period, and what is the refund policy if I cancel?
11. Where will the procedure be done, and can I see the theatre?
12. What will my hairline look like at 50, given my family history?

## 16. Alopecia areata and the JAK inhibitor question

Alopecia areata is the third most common diagnosis in Indian hair clinics — 12.81 percent in the HAILO database. It is an autoimmune condition, not a deficiency, and it behaves completely differently from pattern loss.

The classic picture is one or more completely smooth, round patches with no scaling. Most single patches regrow within a year, sometimes without treatment. Extensive forms — multiple patches, total scalp loss, or loss of eyebrows and eyelashes — need dermatological management: topical or intralesional steroids, contact immunotherapy, and increasingly the JAK inhibitors.

### What has changed

Three oral JAK inhibitors are approved in the United States for severe alopecia areata: baricitinib in June 2022, ritlecitinib in June 2023, and deuruxolitinib in July 2024. In trials they produce substantial regrowth in a meaningful proportion of patients with severe disease, and they are the first real systemic advance in this condition in decades.

In India the picture is more complicated and you should not rely on an influencer's post. Some JAK inhibitors have been available in India for other indications such as rheumatoid arthritis, and dermatologists have used them off-label for severe, treatment-resistant alopecia areata under supervision. Formal Indian approval specifically for the alopecia areata indication, and the availability of the newest molecules, has been evolving; imported brand product has been accessible through the CDSCO named-patient route. Ask your dermatologist what is currently approved and stocked, rather than assuming.

### The honest caveats

- They are immunosuppressive and carry warnings about serious infections, tuberculosis reactivation, blood count changes and raised lipids. Screening before starting is not optional.

- They are expensive, and in India often paid out of pocket because insurers may treat hair loss as cosmetic.
- Stopping usually leads to relapse. This is long-term therapy, not a course.
- They are not for pattern hair loss. Do not let anyone sell you a JAK inhibitor for male balding.

## 17. What does not work, and how the claims are made

This chapter is worth more than its price. The Indian hair-loss market is built on products that cannot legally claim what their advertising implies, and the enforcement record shows exactly which claims keep getting struck down.

### The honest verdicts

| Claim | What the evidence says |
|---|---|
| Biotin regrows hair | Only corrects a genuine deficiency, which is rare. High doses can distort thyroid, ferritin and vitamin D blood tests. |
| Onion juice, garlic paste | One small study is cited endlessly; no robust evidence of meaningful regrowth. Smell and irritation are guaranteed. |
| Rosemary oil | One small comparative trial against 2 percent minoxidil; promising but far too thin to replace treatment. |
| Castor oil, coconut oil, amla, bhringraj | Condition the hair shaft and reduce breakage. Do not stimulate follicles or stop genetic loss. |
| Hair-growth shampoos | Contact time is seconds. No shampoo has been shown to regrow hair. |
| “DHT-blocking” oils and serums | No credible clinical evidence at consumer concentrations. |
| Hair spa, scalp massage | Pleasant, relaxing, may improve feel. No evidence of regrowth. |
| Mesotherapy cocktails, multivitamin injections | Widely sold in India; weak and inconsistent evidence, unregulated formulations. |
| Ayurvedic or herbal “permanent cure” | No product cures pattern loss. Schedule J of the Drugs and Cosmetics Rules bars drugs from claiming to cure baldness or grow new hair. |

### How the law sees it

The Drugs and Magic Remedies (Objectionable Advertisements) Act 1954 prohibits advertisements making false or misleading claims about a drug, with penalties including imprisonment. Baldness is not on its schedule of specified conditions, but the general prohibition on false claims applies, and Schedule J of the Drugs and Cosmetics Rules separately bars claims that a drug prevents or cures baldness or grows new hair.

The Advertising Standards Council of India's code requires special restraint in advertisements addressed to people suffering from “baldness and the like”, so that claims do not exceed what is prudent by accepted medical standards. Its Consumer Complaints Council has upheld complaints against clinics and brands claiming “stem cell” regrowth, “100 percent success”, “guaranteed treatment for baldness”, “treatment without side effects”, and even a shampoo claiming “freedom from hair loss”.

### The ten phrases that should make you leave

- “Guaranteed result” or “100 percent success”.
- “Regrow hair in 30 days” or “in 15 days”.

- “No side effects at all”.
- “Stem cell hair therapy” without a clear explanation of what is being injected.
- “Permanent cure for baldness”.
- “Unlimited grafts” at a flat price.
- “Free hair test” that always ends in a package quotation.
- “Our doctor is abroad, the technicians will do it.”
- “This is ayurvedic, so it is safe” — safety and efficacy are separate questions.
- “Buy today, price increases tomorrow” on a medical decision.

None of this means every clinic is dishonest. It means the burden of asking is on you, and the questions in Chapter 28 cost nothing.

## 18. Twenty beliefs that cost Indians the most money

Each of these is something people say in clinics, on YouTube and at the dinner table. Each one is either false or half-true in a way that leads to the wrong purchase.

| What people believe | What is actually true |
|---|---|
| Washing hair causes hair fall | Wash day reveals shedding that had already happened. Detached resting hairs leave during the wash. |
| Oil treats hair loss | Oil conditions the shaft and reduces breakage. It does not stimulate the follicle or stop genetic loss, and overnight oiling can worsen dandruff. |
| Cutting hair makes it grow thicker | Cut ends feel coarser because they are blunt. Growth happens at the root and is unaffected by the barber. |
| Shaving a baby's head produces thicker hair | No. Hair density is set by follicles, not by shaving. |
| Dandruff causes permanent baldness | Dandruff and seborrheic dermatitis cause reversible shedding and inflammation. Treat them — but they do not scar the follicle on their own. |
| Minoxidil works within a month | The earliest honest expectation is three months; visible results usually take six to twelve. |
| Once you start minoxidil you can never stop | You can stop. The hair it was maintaining goes back to the natural trajectory over three to six months. |
| A 10,000 mcg biotin capsule is the fix | Only if you are deficient, which is rare. High doses can distort thyroid, ferritin and vitamin D blood tests. |
| Finasteride makes every man impotent | In pooled trial data, sexual side effects were reported by 3.8 percent on finasteride versus 2.1 percent on placebo. |
| A transplant is permanent, so medicine is unnecessary | The transplanted hair is permanent. Your remaining native hair keeps thinning without medical treatment. |
| A cheap transplant gives the same result | Graft survival, hairline design and who performs each step decide the outcome. Rs. 8 per graft has to be paid for by someone. |
| PRP is stem cell therapy | PRP is your own platelet concentrate. The ASCI has upheld complaints against “stem cell” regrowth claims as unsubstantiated. |
| A laser helmet regrows a bald head | Evidence supports a modest density gain in pattern loss. It does not revive a slick bald area. |

| What people believe | What is actually true |
|---|---|
| Massage and oil can revive a bald patch | Where follicles have scarred or fully miniaturised, no topical restores them. That is what transplants are for. |
| Hair loss is a man's problem | In the HAILO database of 24,595 Indian patients, about 74.8 percent were women. |
| Monsoon and hard water cause baldness | Seasonal shedding is real and temporary. Hard water affects the hair shaft, not the follicle. |
| A blood test will reveal the cause | Tests exclude and confirm specific deficiencies. The diagnosis comes from examining the scalp and the pattern. |
| It is only stress | Stress is a well-established trigger for shedding. It does not explain progressive pattern loss, and “only stress” is not a diagnosis. |
| Ayurvedic means no side effects | Safety and efficacy are separate questions, and Schedule J of the Drugs and Cosmetics Rules bars any drug from claiming to cure baldness. |
| If nothing works, it is your fate | If six months of correct treatment changes nothing, the diagnosis is wrong or the adherence is. Neither is fate. |

## 19. Four people, four plans, real numbers

Abstract advice becomes usable when you see it applied. These four cases are composites built from the patterns the Indian data describes — not real patients — but every cost figure is a genuine 2026 market range.

### Rohit, 29, Bengaluru, software engineer

Receding temples for two years, thinning at the crown, father bald by 40. Photographs show Norwood 3 vertex. He has already spent Rs. 9,000 on a “regrowth” oil and a laser comb bought at 2 am.

Plan: confirm the pattern with a dermatologist, check ferritin, TSH and vitamin D, start minoxidil 5 percent twice daily and finasteride 1 mg daily, add ketoconazole shampoo two to three times a week and weekly microneedling at 1.5 mm. Nothing else. At month six, compare photographs. A transplant becomes a conversation only if the loss is stable at month twelve and he wants the hairline back.

Realistic twelve-month cost: Rs. 700 to Rs. 1,600 a month for medicines and shampoo, plus two consultations at Rs. 500 to Rs. 2,500 each — roughly Rs. 9,000 to Rs. 24,000 for the year, and a real chance of holding what he has plus modest thickening.

### Meera, 34, Pune, schoolteacher

Diffuse shedding for four months, starting about ten weeks after typhoid. Heavy periods for years. Her parting looks slightly wider but her frontal hairline is intact. Ferritin 12, haemoglobin 10.4.

Plan: this is telogen effluvium on top of iron deficiency anaemia, with early female pattern loss to be confirmed. Treat the anaemia properly and find out why she is losing blood; fix protein intake at breakfast; photographs monthly; reassess minoxidil at the next visit if the pattern component is confirmed. Expect improvement over six to nine months, not six weeks.

Realistic twelve-month cost: Rs. 300 to Rs. 900 a month for iron and, if started, minoxidil; two or three consultations. Under Rs. 15,000 for the year — and the most important part of the treatment is a medical review of the blood loss, not a hair product.

### Anjali, 41, Delhi, marketing manager

Two smooth round patches over eight weeks, and her eyebrows look thinner. No itching, no scaling. She has been advised a “hair spa package” by a salon.

Plan: this needs a dermatologist this week, not a package. Smooth patches with eyebrow involvement suggest alopecia areata, and eyebrow loss is a red flag that also warrants excluding frontal fibrosing alopecia and thyroid disease. Treatment may be topical or intralesional steroids; extensive or resistant disease is where JAK inhibitors enter the conversation, with screening and cost discussed honestly. A transplant is not the answer here, and any clinic that offers one first should be left.

Realistic twelve-month cost: highly variable — from a few thousand rupees for steroid treatment to a substantial monthly outlay if a JAK inhibitor is used. Ask for the annual figure in writing before starting.

### Suresh, 52, Kochi, small business owner

Norwood 6, has wanted a transplant for years, has saved Rs. 2,00,000, and has never taken medical treatment.

Plan: measure donor density first — at Norwood 6 the donor supply relative to the area to cover is the whole question. Realistically 3,500 to 4,500 grafts at Rs. 40 to Rs. 150 each is Rs. 1,40,000 to Rs. 6,75,000, and the outcome will be coverage rather than a full head of hair. He should start medical treatment now, wait six to twelve months so the pattern stabilises and the donor area is assessed honestly, and decide between a well-executed transplant and a deliberate short cut. Both are legitimate choices; the indecision is what costs money.

| Case | Main diagnosis | Year-one spend | What to expect |
|---|---|---|---|
| Rohit, 29 | Androgenetic alopecia | Rs. 9,000 to Rs. 24,000 | Maintenance plus modest thickening; transplant optional later |
| Meera, 34 | Telogen effluvium plus iron deficiency | Under Rs. 15,000 | Substantial recovery over 6 to 9 months once the trigger and deficiency are treated |
| Anjali, 41 | Alopecia areata (to be confirmed) | Wide range | Regrowth is common; specialist management essential |
| Suresh, 52 | Advanced androgenetic alopecia | Rs. 1,50,000 and above if operated | Coverage, not density; donor supply is the limiting factor |

> **The pattern across all four**
>
> In every case the first useful spend was a consultation, and the least useful spend was the thing bought before the diagnosis. Notice also that three of the four did not need a procedure at all in year one.

PART THREE

# The daily system that decides the outcome

Treatments do the work. Habits decide whether the treatment gets a chance. This part is about the eight things you do every week — washing, oiling, styling, eating, sleeping, managing stress, taking your medicine, and measuring honestly — and the Indian-specific details that most advice ignores.

# 20. Wash day: frequency, sulphates and hard water

The most common Indian hair myth is that washing causes hair fall. It does not. Washing causes the hair that had already shed to leave your head in a visible place.

Hair that has entered the resting phase is already detached at the root; it sits in the follicle until mechanical action — combing, washing, tying — carries it away. Wash day shows you three days of shedding at once. That is arithmetic, not damage.

### How often, honestly

There is no universal number. If your scalp is oily, daily or alternate-day washing is fine. If it is dry and you have long, chemically treated hair, two or three times a week is enough. What matters is that the scalp is clean: an oily, flaky scalp with product build-up is a worse environment for follicles than a clean one, and seborrheic dermatitis thrives in it.

### Sulphates, and the real problem

Sulphates are efficient detergents. They can dry the hair shaft, especially in already-damaged or chemically treated hair, but they do not cause hair loss. If your scalp is oily, a sulphate-free shampoo may leave you under-washed. Choose based on your scalp, not on a label trend. What genuinely damages hair is not the shampoo — it is rough towel-drying, brushing wet hair, tight ties and heat.

### Hard water is a real Indian problem

Many Indian cities supply water with high total dissolved solids, calcium and magnesium. The visible consequences are dull, stiff hair, mineral build-up and more breakage. The consequences people worry about — permanent follicle damage — are not established. Practical fixes: a shower filter, a final rinse with filtered or RO water, a clarifying wash once a fortnight, and a leave-in conditioner on the lengths. If your hair feels coated a week after a wash, the water is worth addressing.

### The wash-day sequence that minimises breakage

1. Detangle dry, with fingers or a wide-tooth comb, before you wet the hair.
2. Wet with lukewarm water, not hot.
3. Shampoo the scalp only, with fingertips, not nails. Let the lather rinse through the lengths.
4. Condition from mid-length to ends. Rinse thoroughly.
5. Squeeze water out with a cotton towel — do not rub.
6. Comb with a wide-tooth comb while damp, starting at the ends.
7. Air-dry where possible. If you must use heat, use the lowest setting and keep the dryer moving.

# 21. Oiling, honestly

This is the chapter that will annoy somebody. Oiling is a culturally loved practice in India, and the evidence supports it as a conditioner of the hair shaft — not as a treatment for hair loss.

A 2026 survey of 1,200 people across southern India published in the Indian Dermatology Online Journal found that 80 percent oiled their hair at least occasionally — 35 percent two to three times a week, 25 percent weekly, and 8 percent daily. Forty percent reported problems they associated with oiling: dandruff in 25 percent, itchy scalp in 15 percent, hair fall in 10 percent. Seventy percent still believed oiling was beneficial.

Both things are true. Oil reduces friction, improves manageability, and coconut oil in particular has been shown to reduce protein loss from the hair shaft. What oil does not do is stimulate the follicle, block DHT, or reverse genetic thinning. Dermatologists quoted in mainstream Indian coverage in November 2026 were blunt about it: oiling conditions the shaft and does not directly stimulate follicles, and leaving oil overnight can aggravate dandruff or seborrheic dermatitis, particularly on oily scalps.

### A defensible oiling routine

- Apply to the lengths and ends, not a heavy layer on the scalp.
- Leave it on for 30 to 60 minutes before washing. Overnight is unnecessary and, on an oily or flaky scalp, counterproductive.
- Do not massage vigorously. Friction breaks fragile hair; gentle pressure is enough.
- If you have dandruff, seborrheic dermatitis or an itchy scalp, treat the scalp first and cut the oil to occasional use on the ends.
- Never oil an inflamed, red or infected scalp.

If you enjoy oiling, keep doing it — comfort and ritual have value, and stress reduction is not nothing. Just do not count it as your treatment. The money you spend on a premium “regrowth” oil would buy four months of minoxidil, which has trials behind it.

## 22. Styling, tension and the damage you control

Traction alopecia is the one hair loss that is entirely preventable and entirely your choice. It is also more common in India than most people realise, because the hairstyles that cause it are everyday ones.

Traction alopecia comes from prolonged or repetitive tension on the hair — tight braids, tight buns, extensions, glue, gels used for a slicked-back look, tight night-time wrapping, and combining chemical relaxing with braiding. Early on it is reversible: the hairline thins and regrows once the tension stops. Long term, it scars, and then it does not come back. Dermatology reviews describe a “fringe sign” — fine hairs remaining along the edge of an originally lower hairline — which signals that some permanent loss has already happened. There is no approved drug treatment; management is stopping the tension, sometimes with topical or injected steroids for inflammation, and surgery in late cases.

### The two-second test

If your hairstyle hurts, pulls, or leaves tenderness or small bumps at the hairline, it is too tight. Take it down. If you can see the skin at your temples through the hair when your hair is pulled back, you have already lost density there.

### Heat and chemicals

- Flat irons and blow-dryers at high heat cause bubble formation in the hair shaft and permanent structural damage. Keep heat below 180 degrees Celsius and keep the tool moving.
- Bleach and repeated colouring weaken the shaft. Damage shows up as breakage at mid-length, not as loss at the root — which is why it can look like thinning.
- Chemical straightening and relaxing, followed by tight braids or extensions, is the highest-risk combination.
- Never brush wet hair from the roots down. Start at the ends.
- Silk or satin pillowcases reduce overnight friction; a loose braid is better than a tight one.

# 23. Food, weight and the deficiency trap

No diet regrows hair on a balding scalp. But several diets reliably cause shedding, and Indian eating patterns have two specific weak points: protein and iron.

### Protein first

Hair is protein. A diet low in protein — common in weight-loss regimes, in some vegetarian households where dal is the only protein source, and in intermittent fasting patterns that skip meals entirely — produces diffuse shedding. A rough target for an average adult is about 1 gram of protein per kilogram of body weight per day, more if you are training or recovering from illness. For a 60 kg adult that is roughly 60 grams: about two katoris of dal, a bowl of curd, 100 grams of paneer or fish, and some nuts.

The evidence connecting low protein to shedding is clinical and consistent; the evidence that protein supplements regrow hair in well-fed people is essentially absent. Fix a deficit, do not chase an excess.

### Crash diets are a known cause of shedding

Rapid weight loss — 20 pounds or more in a short period, very low-calorie diets, bariatric surgery, severe restriction after a festival season — is one of the classic triggers the American Academy of Dermatology lists for excessive shedding. The shedding appears two to four months later, which is why people rarely connect the two. If you plan a strict diet, plan it with protein and micronutrients in it.

### The supplements that are worth it, and the ones that are not

| Supplement | Verdict |
|---|---|
| Iron (only if ferritin is low) | Worth treating a genuine deficiency. Do not take iron blindly — excess is harmful. |
| Vitamin D (if deficient) | Correct deficiency; do not expect regrowth from it alone. |
| Vitamin B12 (if low) | Treat the deficiency, especially on long-term vegetarian diets and metformin. |
| Zinc | Only if deficient or losing it through illness. Excess zinc causes copper deficiency. |
| Biotin | Only if deficient, which is rare. Can distort thyroid, ferritin and vitamin D blood tests. |
| Multivitamin “hair growth” blends | Expensive reassurance. No trial shows regrowth in well-nourished people. |
| Collagen drinks | No credible evidence for hair growth. |

### What to actually do

Eat breakfast with protein in it. Keep dal, curd, eggs, paneer, fish or soya in the daily rotation. Add a source of vitamin C with iron-rich foods — iron from plants is poorly absorbed without it, and tea or coffee within an hour of a meal reduces absorption further. Do not drink four cups of chai with your meals and wonder why your ferritin is 12.

# 24. Sleep, stress and what is actually proven

Stress causes shedding. That part is well established. Whether stress causes pattern baldness, and whether meditation regrows hair, are different questions.

Telogen effluvium is triggered by physical and psychological stressors, and the American Academy of Dermatology lists severe stress among the classic causes. Indian post-COVID studies make the same

point: in one series of 72 patients with post-COVID shedding, 70 had moderate to severe perceived stress scores. In a Telangana cohort, higher stress was a significant independent predictor of hair loss alongside female sex and COVID severity.

What follows from this is modest but real. Sleep seven to eight hours where you can. Do not use hair loss as a reason to spiral — the anxiety loop is itself a stressor, and it is why the photo protocol in Chapter 2 matters: it replaces dread with data. Move your body most days. If you are in a genuinely bad patch — grief, a job loss, a separation — treat that as the medical issue it is and get help for it, not just a serum.

There is no evidence that any breathing technique, scalp massage or meditation practice regrows hair in pattern loss. There is reasonable evidence that they improve how you feel while you wait for the treatment to work, which is not a small thing.

## 25. The twelve-month calendar: what should happen when

Most people quit effective treatment at month three because nothing looks different. This is the schedule to hold yourself to instead.

| Month | What to expect | What to do |
|---|---|---|
| 0 | Baseline | Photos, blood tests, diagnosis, start treatment |
| 1 | Possible increase in shedding (the dread shed) | Do not stop. Log it. This often means the drug is working |
| 2 | Shedding settling for most people | Check adherence. Are you really applying it daily? |
| 3 | Earliest possible change; usually nothing visible | Photograph again. Review side effects with your doctor |
| 4 | First honest comparison point: month 1 versus month 4 | Look for reduced shedding and less scalp show, not new hair |
| 6 | Thickening of existing hair; visible improvement in some | This is the point at which to judge a treatment |
| 9 | Consolidation if the regimen is working | Do not add new products out of impatience |
| 12 | Full assessment of the year | Decide what continues, what stops, and whether a procedure is now reasonable |

Two rules for this calendar. First, judge a treatment only at six months or later, unless side effects force an earlier decision. Second, if nothing at all has changed at month six — not shedding, not density, not scalp show — the diagnosis or the adherence is wrong. Go back to the clinician with the photos.

> **The adherence reality check**
>
> In real-world use, the biggest reason minoxidil “fails” is that it is applied two or three times a week instead of daily. Before you conclude a treatment does not work, count the days you actually used it in the last 30. If the number is under 25, the treatment has not failed — it has not been tested.

PART FOUR

# Money, clinics and decisions

Hair loss is one of the few consumer categories in India where a Rs. 300 bottle and a Rs. 3,00,000 package are sold for the same problem, often in the same building. This part is about spending deliberately: how the market is structured, what each budget actually buys, how to choose a clinician, what insurance will and will not pay, and the special situations that need a different plan.

# 26. How the Indian hair-loss market is built

Understanding the business model protects you better than any product review.

### Four kinds of seller

- **Dermatologists and plastic surgeons** — paid per consultation and procedure. Their incentive is procedures, but their training and registration are checkable, and the NMC guidelines bind them.
- **Chain clinics** — paid per package, with targets. The consultation is often free because the package is the product. Quality varies enormously between branches of the same brand.
- **Telehealth and D2C brands** — paid per subscription. Convenient and often good value for straightforward pattern loss; weak for anything that needs a scalp examination.
- **Salons and "hair spa" centres** — paid per session and product. Not medical settings. No one there can diagnose you, and under the NMC guidelines nobody there should be performing a surgical or injectable procedure.

### The three-step sale you will recognise

1. **The free test.** A camera, a magnified image, and a number — "you have lost 43 percent of your density". The number is usually generated by software with no validated calibration.
2. **The urgency.** "If you wait six months the follicles will die." For pattern loss, follicles miniaturise over years, not weeks; genuine urgency exists only in scarring alopecia and rapid alopecia areata.
3. **The package.** Six PRP sessions, a laser helmet, a year of supplements and a shampoo — priced as a bundle so you cannot compare any single item.

None of this is illegal. All of it is easier to resist when you walk in with a written list of questions, your own photographs, and a decision rule: **no package on the first visit, ever**. A clinician who will not let you go home and think is telling you what they think of you as a customer.

# 27. Budgets: what Rs. 500, Rs. 1,500 and Rs. 5,000 a month actually buy

These are real Indian 2026 market ranges, and they are the four plans most people end up choosing between.

| Plan | Monthly cost | What it includes | Honest verdict |
|---|---|---|---|
| Nothing but habits | Rs. 0 to 500 | Gentle handling, protein and iron in the diet, sleep, no tight styles, photos monthly | Enough for pure telogen shedding with a clear trigger. Not enough for pattern loss |
| Basics, done properly | Rs. 500 to 1,500 | Minoxidil 5 percent daily (Rs. 250 to 1,400), ketoconazole shampoo, one dermatologist visit twice a year | The best value in the entire market for pattern loss. This is the plan most people should start with |
| Basics plus oral therapy | Rs. 1,500 to 3,000 | Adds finasteride (men) or low-dose oral minoxidil or spironolactone (women), plus monitoring visits | Where the evidence is strongest for moderate to advanced pattern loss |
| Procedure-led | Rs. 5,000 and above | PRP packages at Rs. 3,000 to 12,000 a session, microneedling sessions, laser devices, or a transplant at Rs. 40 to Rs. 150 per graft | Reasonable only on top of the basics, and only after six months of them |

### The twelve-month arithmetic

A year of proper basic treatment in India costs roughly Rs. 6,000 to Rs. 18,000 — less than a single PRP package at many clinics. A year of PRP packages costs Rs. 12,000 to Rs. 72,000. A transplant for a Norwood 4 pattern costs Rs. 75,000 to Rs. 3,00,000, plus medicines afterwards. Spend in that order, and only move up when the cheaper tier has had a fair trial.

> **The one rule that saves the most money**
>
> Never buy a procedure in the first three months after diagnosis. Medical treatment needs that time to show whether it is working, and a procedure done on an untreated, actively progressing scalp is a procedure you may need again in three years.

## 28. Choosing a clinician: the appointment script

A good first appointment for hair loss takes twenty minutes and involves a magnifier, a pull test, questions about your periods, diet and medicines, and a written plan. If you get six minutes and a brochure, you are in a sales room.

### What to check before you book

- The doctor's registration with the state medical council, and their qualification — MD or DNB dermatology, or MCh or DNB plastic surgery for surgical work.
- Whether the clinic is registered as a clinical establishment in your state.
- Whether the consultation is genuinely independent of a package sale.
- Whether they will do trichoscopy (a magnified scalp examination) rather than only a photograph.

### What to say in the room

Open with the facts, not the worry: “I have been shedding more for five months. It is diffuse, worse on wash days. I had dengue in June. My ferritin was 18 last year. My mother has thinning at the crown.” Then ask the four questions that decide the plan:

1. What is my diagnosis, and what makes you confident about it?
2. Is there anything here that could be permanent if we do not treat it now?
3. What is the plan for the next six months, and what will we measure?
4. Which of these options is cheapest, and why are you not recommending that one first?

Write the answers down in the room. A clinician who is comfortable being written down is usually a clinician worth keeping.

## 29. Insurance, GST and what is actually covered

The short answer is that hair loss treatment is usually treated as cosmetic and therefore excluded. The longer answer is that it depends on the cause, the policy wording, and whether the insurer accepts that the condition is a disease.

Indian health insurance policies commonly exclude cosmetic or aesthetic procedures. Where alopecia is a documented medical condition — severe alopecia areata, scarring alopecia, hair loss from chemotherapy or another treated illness — some policies and some insurers will consider a claim, but many will not, and pre-authorisation is usually required. JAK inhibitors for severe alopecia areata are

frequently paid out of pocket.

Practical steps: read the exclusions page of your policy before you assume anything; ask the insurer in writing, quoting your diagnosis code, whether a specific treatment is covered; keep prescriptions and photographs, because a claim without documentation of medical necessity is a claim that gets rejected. If you have a corporate policy, ask HR whether the group cover treats dermatological conditions as medical.

On tax: cosmetic dermatology services attract GST, and a hair transplant is generally treated as a cosmetic procedure rather than a medical one for input credit purposes. Do not plan your budget around a deduction you have not confirmed with a professional.

# 30. Special situations

### After delivery

Shedding starts around two months, peaks around four, and hair usually regains normal fullness within six to nine months. Focus on iron, protein, sleep and reassurance. If you are still losing heavily at twelve months, get examined — that is no longer ordinary postpartum shedding.

### PCOS

Female pattern hair loss with acne, unwanted facial hair or irregular cycles is a hormonal picture, not a shampoo problem. This needs a gynaecologist or endocrinologist alongside a dermatologist, because treating the hair without treating the androgen excess is treating the symptom.

### Thyroid disease

Both underactive and overactive thyroid cause diffuse shedding. If your TSH is out of range, correcting it is the treatment. Shedding usually improves over several months after the thyroid is stabilised, not immediately.

### Chemotherapy and radiation

Anagen effluvium is rapid and dramatic, and it is expected. Hair usually regrows after treatment ends, though texture and colour can change. Scalp cooling, where available, reduces hair loss in some regimens. Everything in this book about cosmetic treatments should wait until your oncologist says the treatment is finished.

### Teenagers

Shedding in a 15-year-old is usually nutritional, iron-related, stress-related or traction-related, and occasionally the start of pattern loss. It is almost never a reason for a transplant. Get a diagnosis, fix the deficiency, loosen the hairstyle, and re-check in six months. Avoid starting lifelong medication casually in someone whose pattern has not declared itself.

### Men over 50

The same treatments work, but the expectations change: a 55-year-old with a Norwood 6 pattern has less donor hair relative to the area to cover, and medical treatment maintains more than it regrows. Decide what outcome you actually want — coverage, or a tidy short cut — before you spend.

### Beard, moustache and eyebrows

Patchy beard loss is often alopecia areata and can regrow on its own. Eyebrow loss is a red flag that deserves prompt examination, because it can signal frontal fibrosing alopecia, thyroid disease or alopecia areata. Do not treat eyebrows with minoxidil without medical advice — the skin there is thin and the

dosing is different.

PART FIVE

# Tools you can use this week

The last part of this book is the part you print. Ten worksheets, a 90-day plan, and a sources appendix so you can check every claim yourself. Print pages as you need them; the blank rows are meant to be filled in by hand.

# 31. Ten printable worksheets

### Worksheet 1: Seven-day shedding diary

| Day | Hairs counted | Wash day? (Y/N) | Trigger or note |
|---|---|---|---|
| Mon | | | |
| Tue | | | |
| Wed | | | |
| Thu | | | |
| Fri | | | |
| Sat | | | |
| Sun | | | |

Count every hair you shed in a fixed 24-hour window and write the total. Bring this page to your appointment — a week of numbers is worth more than a year of memory.

### Worksheet 2: Monthly photo log

| Month | Date | Front hairline | Mid-scalp | Crown | Temple | Notes |
|---|---|---|---|---|---|---|
| 1 | | | | | | |
| 2 | | | | | | |
| 3 | | | | | | |
| 4 | | | | | | |
| 5 | | | | | | |
| 6 | | | | | | |

### Worksheet 3: Treatment adherence tracker

| Week | Minoxidil days (of 7) | Oral medicine days | Microneedling done | Shampoo used | Side effects |
|---|---|---|---|---|---|
| 1 | | | | | |
| 2 | | | | | |
| 3 | | | | | |
| 4 | | | | | |
| 5 | | | | | |
| 6 | | | | | |

If minoxidil days are under 25 out of 30, the treatment has not failed. It has not been tested.

### Worksheet 4: Blood test tracker

| Test | Date | Result | Lab range | Target we agreed | Action |
|---|---|---|---|---|---|
| Haemoglobin | | | | | |
| Ferritin | | | | | |
| TSH | | | | | |
| Vitamin D | | | | | |
| Vitamin B12 | | | | | |
| Other | | | | | |

### Worksheet 5: Clinic question sheet

1. What is my diagnosis, and what makes you confident about it?
2. Is anything here permanent if we wait?
3. What is the six-month plan, and what will we measure?
4. What is the cheapest option, and why are you not recommending it first?
5. What is the total cost this year, including everything excluded from the quote?
6. Who performs each step of the procedure, and what is their registration number?
7. What happens if it does not work?
8. When do we review, and what would make you change the plan?

### Worksheet 6: Twelve-month cost planner

| Item | Monthly cost | Months | Year total | Keep or drop at review |
|---|---|---|---|---|
| Minoxidil | | 12 | | |
| Oral medicine | | 12 | | |
| Shampoo | | 12 | | |
| Supplements | | | | |
| Consultations | | | | |
| PRP or procedures | | | | |
| Microneedling device | | | | |
| Total | | | | |

### Worksheet 7: Red flag checklist

| Sign | Yes | No | What it means |
|---|---|---|---|
| Smooth patch with no hair openings | | | Alopecia areata until proven otherwise |
| Eyebrow or eyelash loss | | | Needs prompt examination |
| Shiny, scarred or pale scalp | | | Possible scarring alopecia — urgent |
| Pain, burning or tenderness | | | Inflammation — needs review |
| Pus, crusting or pustules | | | Infection — needs treatment |
| Sudden handfuls of hair | | | Not ordinary shedding |

| Sign | Yes | No | What it means |
|---|---|---|---|
| Fever, joint pain, ulcers with hair loss | | | Systemic — see a doctor |

### Worksheet 8: Product ingredient decoder

| On the label | What it is | Worth paying extra for? |
|---|---|---|
| Minoxidil 5 percent | The only OTC drug proven for pattern loss | Yes — this is the active ingredient that matters |
| Ketoconazole 2 percent | Anti-fungal with mild anti-androgen effect | Yes, as a scalp adjunct |
| Redensyl, Procapil, caffeine, peptides | Cosmetic actives | Unproven for regrowth; fine if you like the formulation |
| Biotin 10,000 mcg | A vitamin you almost certainly do not lack | No — and stop it before blood tests |
| “DHT blocker” on a cosmetic | Marketing language | No |
| Sulphate-free | Milder detergent | Only if your scalp is dry |

### Worksheet 9: Should I get a transplant scorecard

| Question | Yes | No |
|---|---|---|
| Have I been on medical treatment for at least 6 to 12 months? | | |
| Is my loss stable rather than rapidly progressing? | | |
| Do I have good donor density at the back and sides? | | |
| Have I had a scalp examination by a qualified doctor? | | |
| Do I understand that density, not a teenage hairline, is the goal? | | |
| Can I afford the full cost including medicines and follow-up? | | |
| Have I checked the surgeon's registration and seen the theatre? | | |
| Am I prepared to stay on medical treatment afterwards? | | |

Six or more “yes” answers means a transplant is a reasonable conversation. Fewer than six means the conversation is premature.

### Worksheet 10: The 90-day plan

| Days | Do this | Done |
|---|---|---|
| 1 to 7 | Seven-day shedding diary, four baseline photos, book a dermatologist | |
| 8 to 14 | Blood tests (stop biotin 48 hours before), buy nothing else yet | |
| 15 to 30 | Diagnosis appointment with the question sheet, start the prescribed basics | |
| 31 to 60 | Adherence only. Log side effects. No new products | |
| 61 to 90 | Photos, review, decide what continues into month 4 to 12 | |

## 32. The one-page summary

If you remember nothing else from this book, remember these nine things.

1. Shedding and loss are different problems. Get the diagnosis before you buy anything.
2. Normal shedding is 50 to 100 hairs a day. What matters is a widening part, a moving hairline or a visible scalp.
3. In India, telogen effluvium is the most common diagnosis and women are the majority of patients — so most online advice is aimed at the wrong person.
4. Photograph on the first of every month. Memory is unreliable; photographs are not.
5. For pattern loss, minoxidil is the baseline, finasteride is the strongest lever for men, and microneedling is the best-value add-on.
6. Judge nothing before six months, and count your adherence before you blame the treatment.
7. Oiling conditions hair. It does not treat hair loss. Never oil an inflamed scalp.
8. Nothing sold as a “cure” cures anything. Schedule J of the Drugs and Cosmetics Rules bars such claims for drugs, and the ASCI code bars exaggerated ones in advertising.
9. Spend in tiers: habits, then basics, then oral therapy, then procedures. Never buy a procedure in the first three months.

**A note on what this book is**

This is an educational guide. It is not a prescription, a diagnosis, or a substitute for examination by a qualified doctor. Every dose, drug and procedure mentioned here needs a clinician who has looked at your scalp, knows your history and your other medicines, and takes responsibility for the decision. What this book is for is making that conversation informed, shorter and cheaper.

# Appendix: sources, and how to check them

Every number in this book comes from one of the sources below. Where a figure is a market range rather than a published statistic, the text says so. Where the evidence is weak, the text says that too.

### Clinical guidance and patient information

- American Academy of Dermatology — patient pages on hair shedding versus hair loss, and on diagnosis and treatment of hair loss (normal shedding of 50 to 100 hairs a day; telogen effluvium timing; minoxidil results at 6 to 12 months; the effect of stopping).
- Mayo Clinic — patient guidance on hereditary hair loss and minoxidil.
- Indian Journal of Dermatology, Venereology and Leprology — guidelines on the use of finasteride in androgenetic alopecia.
- National Medical Commission — Guidelines on Aesthetic Surgery and Hair Transplant Procedures, 20 September 2022.
- Tamil Nadu Health and Family Welfare Department — Government Order regulating aesthetics, cosmetology and hair transplant clinics, February 2026.
- Advertising Standards Council of India — Code for Self-Regulation of Advertising Content, and Consumer Complaints Council recommendations covering hair-loss claims.
- Drugs and Magic Remedies (Objectionable Advertisements) Act 1954; Schedule J of the Drugs and Cosmetics Rules.

### Indian epidemiological studies

- HAILO multicentre database study of 24,595 Indian patients with hair loss — distribution of telogen effluvium, female pattern hair loss, alopecia areata and male pattern hair loss.
- Population-based study of 1,005 Indian men aged 30 to 50 on the prevalence and pattern of androgenetic alopecia.
- Clinic-based study of female pattern hair loss in western India and its association with metabolic syndrome.
- Indian Dermatology Online Journal 2026 — survey of hair-oiling practices and scalp symptoms among 1,200 respondents in southern India.
- Post-COVID alopecia studies from Telangana and central India reporting prevalence and timing of shedding after infection.

### Treatment trials and meta-analyses

- Dhurat et al., International Journal of Trichology 2013 — randomised evaluator-blinded study of microneedling in androgenetic alopecia.
- 2025 systematic review and meta-analysis of combined microneedling therapy versus topical minoxidil (12 randomised trials, 631 patients).
- International expert consensus on low-dose oral minoxidil for hair loss, 2024–2025 (43 specialists, 12 countries).
- JAMA Dermatology 2024 — randomised trial of oral versus topical minoxidil in male androgenetic alopecia.
- Meta-analysis of oral versus topical minoxidil in androgenetic alopecia, 2024; and dose-response meta-regression of low-dose oral minoxidil.
- JAMA Dermatology 2010 — systematic review of finasteride efficacy and safety in androgenetic alopecia; and the phase III programme data on 1,879 men.
- JAMA Dermatology 2015 — meta-analysis of adverse event reporting in finasteride trials.
- 2025 systematic review and meta-analysis of platelet-rich plasma in alopecia (43 randomised trials, 1,877 participants); and the 2024 meta-analysis of PRP with minoxidil.
- 2024 systematic review and meta-analysis of low-level laser and LED therapy in alopecia (38 studies, 3,098 patients); and the 2024 comparison of minoxidil with and without laser therapy.
- Reviews of biotin for hair loss and the US FDA safety communications on biotin interference with laboratory immunoassays.
- Clinical reviews on serum ferritin thresholds in hair loss, and on laboratory testing in telogen effluvium.
- JAMA Dermatology 2023 — review of traction alopecia.
- Published data on baricitinib, ritlecitinib and deuruxolitinib approvals for severe alopecia areata, and on Indian access through the CDSCO named-patient route.

### Prices

All rupee figures are indicative Indian market ranges for 2026, drawn from publicly listed pharmacy prices and clinic price lists, and rounded. Medicine prices vary by brand, city, pack size and pharmacy discount. Procedure prices vary by city, technique, graft count and surgeon seniority. Treat every figure as a starting point for a quotation, not as a rate card.

### How to check anything in this book yourself

1. For a treatment claim, search the treatment name plus “randomised” or “meta-analysis” on PubMed, and read the abstract's conclusion, not the press release.

2. For a drug's Indian status, check the CDSCO approved-new-drugs list rather than a clinic blog.
3. For an advertising claim, check the ASCI Consumer Complaints Council recommendations — upheld complaints are published.
4. For a doctor's credentials, check the state medical council registration.
5. For a price, ask three clinics in writing for an itemised quotation including exclusions.

### You now know more than the advertisement does.

The sequence from here is simple. Seven days of counting. Four photographs. One honest appointment with a registered dermatologist and a written question sheet. Six months of consistent, cheap, evidence-based treatment before you consider anything expensive. And a monthly photograph so that you, not a clinic's software, decide whether it is working.

Most people who feel they are losing the fight are not losing it because the treatments do not work. They are losing it because nobody told them what they had, how long it would take, and what to stop buying. That is now fixed.

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